

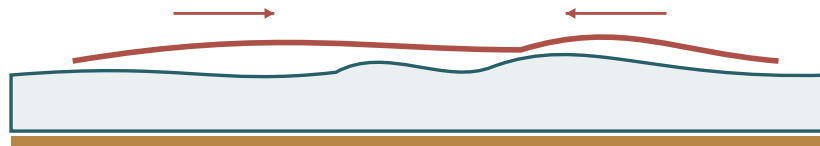
Macular traction: read the interface on OCT

Clinical M3-M5 | Focused Type B notes

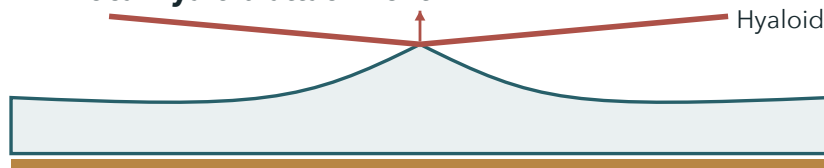
Three different shapes, three mechanisms

Surface contraction, hyaloid pull or a full-thickness gap

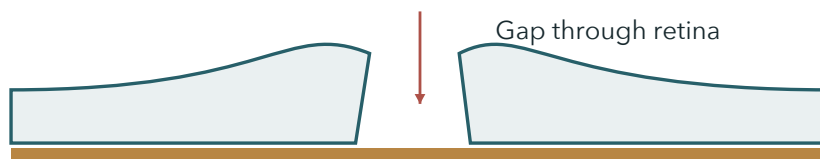
ERM: tangential surface traction



VMT: focal hyaloid attachment



Full-thickness macular hole



Original structural schematics; not patient OCT scans

Original explanatory cross-sections, not patient OCT scans. Use OCT to define the tissue relationship before choosing a treatment.

Distortion is a symptom, not the diagnosis

Finding	Inference
Wrinkled inner surface membrane	Epiretinal membrane; tangential pull.
Focal attached hyaloid + deformed fovea	Vitreomacular traction.
Full-thickness foveal discontinuity	Macular hole.

Inspect the full scan

Check the inner retinal surface, the posterior hyaloid and continuity through the fovea. Cystic spaces can accompany traction; their presence does not automatically establish vascular leakage needing anti-VEGF.

References ASRS: Epiretinal membranes | AAO: Vitreomacular traction | ASRS: Macular hole

Treatment follows symptoms and structure

Select the next discussion

Pattern	Direction
Mild stable ERM/VMT	Monitor function and serial OCT.
Progressive disabling distortion or structural threat	Vitreoretinal assessment.
Symptomatic surgically suitable full-thickness hole	Discuss vitrectomy-based repair, including gas/posturing and surgical risks if proposed.

Procedure pitfalls

Misstep	Why it misses the cause
Anti-VEGF for every cyst	Tractional deformation is not the same as neovascular leakage.
Peripheral laser for a macular hole	It does not release foveal traction or close the central defect.
Diagnose completed PVD despite foveal pull	A persistent focal attachment means the separation is incomplete.

Keep the peripheral retina in view

A macular explanation for distortion does not cancel assessment of new flashes, floaters or a curtain. Macular OCT and peripheral retinal examination answer different clinical questions.